

MEMBER FAQ & GLOSSARY OF INSURANCE TERMS

SilverShield Health Plans | Plan Year 2025

FREQUENTLY ASKED QUESTIONS

Q: How do I find an in-network doctor?

Visit silvershieldhealthplans.com and use the 'Find a Doctor' tool. You can search by specialty, location, language spoken, and whether they are accepting new patients. You may also call Member Services at 1-800-555-0100. Always confirm a doctor's network status by calling their office before scheduling an appointment, as the provider directory is updated monthly but may not reflect real-time changes.

Q: Do I need a referral to see a specialist?

No. SilverShield is a PPO plan and does not require referrals to see in-network specialists. However, some specialist services require prior authorization (PA) before you receive them. Your specialist's office should handle the PA request, but you should verify this before your appointment. A list of services requiring PA is provided in the Prior Authorization Requirements document.

Q: What is the difference between my deductible, copay, and coinsurance?

These are three different ways you share in the cost of your health care:

DEDUCTIBLE: The amount you must pay out-of-pocket each year BEFORE the plan starts paying for most covered services. Example: If your deductible is \$3,500, you pay the first \$3,500 of covered medical bills each year.

COPAY: A fixed dollar amount you pay for a specific service, regardless of the deductible status. Example: A \$25 copay for a primary care visit means you pay \$25 every time you see your PCP, even before the deductible is met.

COINSURANCE: After you have met your deductible, you and the plan share costs as a percentage. Example: If your coinsurance is 20%, you pay 20% of the allowed amount and the plan pays 80%.

Q: What is an out-of-pocket maximum and how does it protect me?

The out-of-pocket maximum (OOP Max) is the most you will ever pay in a plan year for covered in-network services. Once you reach this limit, SilverShield pays 100% of covered in-network services for the rest of the year. For 2025: Bronze Core individual OOP Max is \$8,700; Silver Plus is \$6,000; Gold Premier is \$4,000. Your deductible, copays, and coinsurance all count toward your OOP Max. Premiums do NOT count.

Q: How do I fill a prescription through mail order?

SilverShield Mail Order Pharmacy allows you to get up to a 90-day supply of maintenance medications for a lower copay than 30-day retail fills. To enroll: (1) Ask your doctor to write a 90-day prescription; (2) Create an account at silvershieldhealthplans.com/mailorder; (3) Upload or mail the prescription; (4) Medications are delivered to your home within 7–10 business days. Specialty medications are not available through standard mail order; contact SilverShield Specialty Pharmacy at 1-800-555-0500.

Q: What do I do if I receive a bill from a provider?

First, wait for your Explanation of Benefits (EOB) from SilverShield. Compare the EOB with the provider bill to ensure they match. If they do, pay only the amount shown as your responsibility on the EOB. If you believe there is an error, call Member Services at 1-800-555-0100 before paying. Common billing errors include: duplicate billing, incorrect member ID, incorrect procedure code, or billing for a service that should have been covered.

Q: How does prior authorization work?

Prior authorization (PA) is a process where your doctor gets advance approval from SilverShield before you receive certain services or medications. Your doctor or their office typically initiates the PA request. SilverShield reviews the clinical information and responds within the timeframes specified in the Prior Authorization Requirements document. If PA is approved, the service is covered at your normal cost-sharing level. If PA is denied, you can appeal the decision. Services received without required PA may be denied.

Q: Can I use out-of-network providers?

Yes, SilverShield covers out-of-network services, but your cost-sharing will be significantly higher. Out-of-network providers have not agreed to SilverShield's negotiated rates, meaning you may also be responsible for the difference

between what they charge and the SilverShield allowed amount (balance billing). Emergency care is always covered at in-network rates, regardless of provider network status. For non-emergency care, using in-network providers will always result in lower costs.

Q: What mental health services are covered?

SilverShield covers mental health and substance use disorder services at parity with medical benefits (required by federal law). Covered services include: outpatient therapy (individual and group), psychiatric medication management, intensive outpatient programs (IOP), partial hospitalization programs (PHP), inpatient psychiatric care, and substance use disorder treatment (detox, residential, outpatient). Applied Behavior Analysis (ABA) for autism is covered with no visit limit. Prior authorization is required for inpatient psychiatric admissions and residential treatment programs.

Q: What happens if I have an emergency while traveling?

Emergency care is covered at in-network rates anywhere in the United States. Go to the nearest emergency room. SilverShield's emergency care benefit covers treatment for emergency medical conditions regardless of provider network status. After the emergency, follow-up care should be transitioned to an in-network provider as soon as medically feasible. For international travel, emergency care outside the U.S. is generally NOT covered, except in limited circumstances specified in your plan. SilverShield recommends purchasing travel insurance for international trips.

GLOSSARY OF KEY HEALTH INSURANCE TERMS

Allowed Amount

The maximum payment the plan will pay for a covered health care service. May also be called 'eligible expense,' 'payment allowance,' or 'negotiated rate.' If your provider charges more than the allowed amount, you may owe the difference (balance billing) if using an out-of-network provider.

Balance Billing

When a provider bills you for the difference between their charge and the allowed amount. In-network providers cannot balance bill you (they have agreed to accept the allowed amount). Out-of-network providers may balance bill you, which is why in-network care is generally less expensive.

Benefit Period / Plan Year

The 12-month period during which your benefits apply. For SilverShield employer plans, this is typically January 1 through December 31. Deductibles, out-of-pocket maximums, and visit limits reset at the beginning of each plan year.

Certificate of Coverage

A legal document that describes your health plan benefits in detail. The Certificate of Coverage governs your plan; if there is a conflict between the Summary of Benefits and Coverage and the Certificate of Coverage, the Certificate controls.

COBRA (Consolidated Omnibus Budget Reconciliation Act)

A federal law that gives you the right to continue your employer-sponsored health coverage for a limited time after leaving a job, reducing work hours, or experiencing other qualifying events. COBRA coverage is typically available for 18–36 months and requires you to pay the full premium plus a 2% administrative fee.

Coinsurance

Your share of the costs of a covered service, calculated as a percentage of the allowed amount after the deductible is met. Example: If your coinsurance is 20% and the allowed amount for a service is \$500, you pay \$100 and the plan pays \$400.

Copayment (Copay)

A fixed dollar amount you pay for a covered service, usually at the time of service. Copays for primary care, specialist visits, and prescription drugs are typically charged before meeting the deductible. Copay amounts vary by service type and plan tier.

Cost-Sharing

Any combination of deductibles, copays, and coinsurance that you pay for covered health care services. Premiums are not considered cost-sharing.

Deductible

The amount you pay out-of-pocket for covered health care services before your insurance plan starts to pay. For family coverage, there is both an individual deductible and a family deductible. The family deductible is met when combined family expenses reach the family deductible amount, even if no single individual has met the individual deductible.

Explanation of Benefits (EOB)

A statement sent by SilverShield after processing a claim, explaining what was billed, what the plan paid, and what you owe. An EOB is not a bill. Keep EOBs for your records and compare them to provider bills.

Formulary

A list of prescription drugs covered by SilverShield, organized into tiers (Generic, Preferred Brand, Non-Preferred Brand, Specialty). Drugs on the formulary are covered; drugs not on the formulary may require a formulary exception to be covered. The formulary is updated periodically; check silvershieldhealthplans.com/formulary for the current list.

Health Savings Account (HSA)

A tax-advantaged savings account available to members enrolled in a High-Deductible Health Plan (HDHP). Funds contributed to an HSA can be used tax-free for qualified medical expenses. SilverShield's Bronze Core plan is HSA-eligible. 2025 HSA contribution limits: \$4,300 (individual), \$8,550 (family).

In-Network

Providers (doctors, hospitals, labs, etc.) who have contracts with SilverShield to provide services at negotiated rates. Using in-network providers results in lower cost-sharing for you. Always verify a provider's network status before receiving services.

Medically Necessary

Health care services or supplies needed to diagnose or treat an illness, injury, condition, disease, or its symptoms, consistent with generally accepted standards of medical practice. SilverShield's Medical Director reviews clinical information to determine medical necessity when required.

Network

The group of doctors, hospitals, and other health care providers who have a contract with SilverShield to provide services to members at negotiated rates. SilverShield's PPO network includes thousands of providers nationwide.

Out-of-Network

Providers who do not have a contract with SilverShield. You can still use out-of-network providers, but your cost-sharing will be higher and you may be subject to balance billing (the provider can charge the difference between their fee and SilverShield's allowed amount).

Out-of-Pocket Maximum (OOP Max)

The most you will pay in a plan year for covered in-network services. Once you reach this limit, SilverShield pays 100% of covered in-network services for the rest of the year. The OOP Max includes deductibles, copays, and coinsurance but not premiums.

Pre-Existing Condition

A health condition that existed before the start of a new health plan. Under the Affordable Care Act, SilverShield cannot deny coverage or charge higher premiums based on pre-existing conditions.

Premium

The monthly amount you pay for health insurance coverage, regardless of whether you use medical services. Employer-sponsored premiums are typically split between you and your employer. Premiums do not count toward your deductible or out-of-pocket maximum.

Prior Authorization (PA)

Approval from SilverShield required before certain services or medications are covered. Your provider typically requests PA on your behalf. See the Prior Authorization Requirements document for a complete list of services

requiring PA.

Step Therapy

A process requiring you to try one or more specified drugs before a higher-cost drug is covered. For example, SilverShield may require you to try a generic metformin before covering a GLP-1 receptor agonist for Type 2 diabetes. Step therapy requirements are specified in the formulary.

Summary of Benefits and Coverage (SBC)

A standardized, easy-to-understand document that summarizes your health plan's benefits and costs. The SBC allows you to compare plans during open enrollment. By law, SilverShield must provide the SBC before enrollment and upon request.

Urgent Care

Medical care for conditions that are not life-threatening but require prompt attention (within 24 hours). Urgent care centers are an alternative to emergency rooms for non-emergency conditions and are typically less expensive. SilverShield covers in-network urgent care visits at a lower copay than emergency room visits.

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